

REQUEST FOR MEDICAL INFORMATION — FORM B — Treating Psychiatrist

Questions arising from the correspondence of Metro South Health dated 31 July 2026 · Ref MSH-INJ-5795 · CLM-317073

Patient: Mr Cory Shepherd **DOB:** 11 January 1991
Employer: Metro South Health — Logan and Beaudesert Health Service
Role: Administration Officer (AO3), Switchboard Services, Logan Hospital (permanent full-time, 76 hours per fortnight)

To: Dr Ravikumar Bangalore Krishnaiah
Consultant Psychiatrist
Mind and Memory Service
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info@mindandmemoryservice.com.au

Date: 31 July 2026

This form is provided by the patient, Mr Cory Shepherd. It is not a document of Metro South Health. The Health Service issued nine questions on 31 July 2026. Four of those are not medical questions and have been returned to the Health Service. The questions below are those properly within your field. They are reproduced as the Health Service put them.

Metro South Health has advised in writing that it "will meet your reasonable costs of preparing this report". Invoices are to be sent to Ms Michelle Harrison, Injury Management Consultant, Human Resources, Logan and Beaudesert Health Service, lbh_InjuryManagement@health.qld.gov.au.

Questions for Dr Ravikumar Bangalore Krishnaiah, treating psychiatrist

Mr Shepherd's object is a safe and sustainable return to his substantive role. The Health Service has asked what workplace controls are medically necessary. Your opinion is sought on that question, and on the functional matters below, so that the workplace can be adjusted around them. A holistic formulation is welcomed, including Mr Shepherd's own understanding of his condition and its management.

1(d) Whether exposure to the identified workplace stressors presents a foreseeable risk to Mr Shepherd's health or safety if he were to return to work, and if so, what specific workplace controls or adjustments you consider medically necessary to manage that risk.

Mr Shepherd notes that identifying and implementing controls is a function of the employer under Part 3.1 of the Work Health and Safety Regulation 2011. Your opinion is sought on what is medically necessary.

7 Your report of 13 February 2025 states that Mr Shepherd's "working memory is affected under stress". Please clarify what this means in functional terms in the workplace, including (a) how this may affect capacity to perform substantive duties under normal workplace demands, and (b) the types of circumstances likely to cause this difficulty.

8 Are there specific tasks, situations or environments (for example high-pressure settings, shift work, patient acuity) that may exacerbate Mr Shepherd's condition or symptoms?

The role is a continuous shift working role covering multiple shifts over a 24/7 period.

- 9 Mr Shepherd reports that distress rises as a rostered shift approaches and peaks at the point of entering the workplace, and that this has resulted in lateness and in late notice of absence. Could you please address, in functional terms, whether distress of that kind in anticipation of attendance is a feature of his condition, and what its effect on attendance and punctuality would be expected to be.**

- 10 Over the twelve months preceding the Employee Capabilities Checklist Mr Shepherd was rostered and worked in the order of six shifts per fortnight, a pattern the Checklist records as having been “worked and tolerated... without deterioration”. In your opinion, is a pattern of that kind sustainable for him, and what rostered pattern would you consider appropriate going forward?**

The Health Service set and accepted that roster over the period. Where his condition was exacerbated and he could not attend a rostered shift, Mr Shepherd managed that himself by taking sick leave or annual leave; no further adjustment was sought or provided, and his paid sick leave is now exhausted. Your opinion is sought on what is sustainable, not on what has been rostered.

- 11 Having regard to your answer to question 9, Mr Shepherd has raised for consideration a sequence of (i) a telephone call one hour before each rostered shift to confirm attendance, (ii) a second call if the first is not answered or attendance is not confirmed, and (iii) engagement of the on-call operator to cover the shift if attendance is still not confirmed; together with an escalation pathway he may initiate during a shift if acutely exacerbated. In your opinion, would arrangements of that kind be clinically appropriate, and are there others you would recommend in preference?**

These are offered for consideration only. Items (i) to (iii) are rostering measures and require no colleague to know anything about his health. He has expressly declined any arrangement under which colleagues observe or assess his condition. Your opinion on what is medically necessary is what the Health Service has asked for.

This report is sought in respect of current capacity, functional restrictions and workplace controls. It is not sought on the aetiology of the injury, which is a separate matter and is the subject of separate instructions. The purpose of this report is collaborative. Mr Shepherd wishes to return to his substantive role and is seeking the identification of workplace controls that would allow him to do so safely and sustainably. A holistic formulation is welcomed, including the patient's own understanding of his condition, its triggers and its management, which Mr Shepherd regards as central to working safely. Nothing in this request should be read as inviting an adverse or medico-legal opinion about any person.

Practitioner name		Provider no.	
Signature		Date	
Practice details / stamp			